To build this public application for the AAPI community, you can integrate the legislative focus on **diabetes screening thresholds** and **language access** with the funding details from the **AAPI Equity Budget**.
Here is how you can specifically implement the "Diabetes Prevention & Screening Tool" using the information in the sources:

1. The AAPI-Specific Risk Calculator

The core of your tool would be based on the proposed legislation addressing the **"shocking rise" in diabetes** among Asian Americans 1, 2.

- **Source Implementation:** While the general medical standard for screening might be higher, your app would use the **lower Body Mass Index (BMI) thresholds** advocated for in the new legislation to flag risk earlier for AAPI users 1, 2.
- **User Value:** This provides a culturally tailored health assessment that accounts for the fact that AAPI individuals often develop diabetes at lower weights than other populations 1.

2. Interactive Neighborhood Mapping (Open Data)

To visualize the need, you would utilize the **NYC Open Data API** to fetch the **"Community Health Survey"** or **"Diabetes Prevalence"** datasets.

- **Data Integration:** By querying these datasets, you can identify neighborhoods with the highest concentrations of diabetes and overlap them with areas where AAPI populations are growing the fastest (currently **over 11% of New York State**) 3, 4.
- **Feature:** The map would highlight "High-Risk Zones" where systemic disinvestment has historically hindered health outcomes 3, 5.

3. Culturally Competent Provider Directory

A critical feature is connecting at-risk users to the **150+ non-profits** funded by the **$54.35 million AAPI Equity Budget** 3, 4, 6.

- **Source Implementation:** You can feature organizations like the **South Asian Council for Social Services (SACSS)**, which serves over 300,000 New Yorkers and provides support in over **20 different languages**, including Hindi, Bengali, Urdu, Cantonese, and Mandarin 7.
- **Language Access Integration:** The app would indicate which providers are compliant with the new **Hospital Language Access Law (S6288B/A387B)**, ensuring they have **Language Assistance Coordinators** and skilled interpreters 8-10.

4. Language-Accessible Navigation

Since **43% of Asian Americans in NYC** have Limited English Proficiency (LEP), the application itself must be multilingual to be effective 11.

- **Source Implementation:** One source notes that 87% of AAPI individuals enrolled in health insurance through certain community centers do not speak English as a primary language 12, 13.

- **App Feature:** Your tool should mirror the translation efforts of the State, which provides critical health information in languages such as **Bengali, Chinese, Korean, and Urdu** 14. This prevents the burden from falling on young children to translate complex medical data for their parents 15-17.

Summary of App Architecture:

- **Backend:** Uses the **NYC Open Data API** to pull real-time health and demographic trends.
- **Logic:** Applies the **lower BMI screening criteria** from the AAPI-focused diabetes bill 2.
- **Frontend:** A multilingual map and directory of the **150+ CBOs** (like SACSS and MinKwon Center) receiving state equity funding to provide culturally responsive care 3, 7, 13.

By merging these datasets, your application would move beyond a simple map to become a tool for **health equity**, ensuring that the most vulnerable New Yorkers can access life-saving screenings in their own language 11, 18.